

Redacted

PAGE 1

PATIENT:

REG DR:

AGE/SX:

DOB:

STATUS:

ROOM:

BED:

TLOC:

REG:

DIS:

COLL:

TIME IN FORMALIN: 4:16 hrs.

CLINICAL INFORMATION:

Pre-Op Diagnosis: Right breast CA

Remarks:

- Specimen(s):
- A. Right breast tissue stitch axillary tail
 - B. Right axillary node x2
 - C. #1 sentinel node right breast
 - D. IS level 3 node stitch marks highest level node
 - E. Axillary contents

*Carcinoma, Lobular infiltrating 8520/3
Site C4CF @ Breast NOS C50.9
path @ Breast, upper-outer quadrant C50.4
JAD 4/2/13*

MICROSCOPIC DIAGNOSIS

- A. RIGHT BREAST, TOTAL MASTECTOMY:
- INVASIVE LOBULAR CARCINOMA
 - INVASIVE CARCINOMA MEASURES AT LEAST 4.5 CM IN GREATEST DIMENSION (pT2)
 - NOTTINGHAM COMBINED HISTOLOGIC GRADE 2 OF 3
 - TUBULE FORMATION SCORE 3 OF 3
 - NUCLEAR PLEOMORPHISM SCORE 2 OF 3
 - MITOTIC COUNT SCORE 1 OF 3
 - SURGICAL MARGINS OF RESECTION FREE OF INVASIVE LOBULAR CARCINOMA
 - FOCI OF LOBULAR CARCINOMA IN SITU
 - FOCI OF DUCT CARCINOMA IN SITU, INTERMEDIATE NUCLEAR GRADE, NOT EXTENSIVE
 - SEE COMMENT FOR SURGICAL PATHOLOGY CANCER CASE SUMMARY CHECKLIST
- B. RIGHT AXILLARY LYMPH NODES X2, EXCISION:
- METASTATIC BREAST CARCINOMA IN TWO OF TWO LYMPH NODES
- C. SENTINEL LYMPH NODE #1, EXCISION:
- NEGATIVE, NO EVIDENCE OF METASTATIC CARCINOMA IN THIS SPECIMEN
- D. LEVEL 3 AXILLARY LYMPH NODES, REGIONAL RESECTION:
- NEGATIVE, NO EVIDENCE OF METASTATIC CARCINOMA IN SIX HIGHEST LEVEL 3 LYMPH NODES
- E. RIGHT AXILLARY LYMPH NODES, REGIONAL RESECTION:
- METASTATIC BREAST CARCINOMA IN ONE OF 14 AXILLARY LYMPH NODES

COMMENT(S)

SURGICAL PATHOLOGY CANCER CASE SUMMARY CHECKLIST - CAP APPROVED

Procedure:

Total mastectomy

** CONTINUED ON NEXT PAGE **

RUN DATE:
RUN TIME:
RUN USER:

PAGE 2

SPEC #:

PATIENT:

(Continued)

COMMENT(S)

(Continued)

Lymph Node Sampling: Sentinel lymph node(s), axillary dissection
Specimen Laterality: Right
Histologic Type of Invasive Carcinoma: Invasive lobular carcinoma
Tumor Size: Greatest dimension of largest focus of invasion: 45 mm
Histologic Grade (Nottingham): Glandular/Tubular Differentiation: Score 3
Nuclear Pleomorphism: Score 2
Mitotic Rate: Score 1
Overall Grade: Grade 2
Ductal Carcinoma In Situ: DCIS is present; negative for extensive intraductal component (EIC)
Architectural Patterns: Cribriform, solid
Nuclear Grade: Grade II
Necrosis: Present, focal
Lobular Carcinoma In Situ: Present
Margins: Invasive Carcinoma: Margins uninvolved by invasive carcinoma
Distance from closest margin: 10mm
DCIS: Margins uninvolved by DCIS
Distance from closest margin: >10 mm
Lymph Nodes: Number of sentinel lymph nodes examined: 1
Total number of lymph nodes examined (sentinel and nonsentinel): 23
Number of lymph nodes with macrometastases (>2 mm): 3
Number of lymph nodes with micrometastases (>0.2 mm to 2 mm and/or >200 cells): 0
Number of lymph nodes with isolated tumor cells (≤ 0.2 mm and ≤ 200 cells): 0
Number of lymph nodes without tumor cells identified: 20
Extranodal Extension: Not identified
Method of Evaluation: Hematoxylin and eosin, level 1
Lymph-Vascular Invasion: Not identified
Dermal Lymph-Vascular Invasion: Not identified
Pathologic Staging: Primary Tumor: pT2
Regional Lymph Nodes: pN1a
Distant Metastasis: Not applicable
Ancillary Studies: ER: Positive (92% of tumor cells with nuclear positivity); strong
PR: Negative
HER2: Positive (score 3+)
In Situ Hybridization for HER2: Not amplified

** CONTINUED ON NEXT PAGE **

RUN DATE:
RUN TIME:
RUN USER:

Specimen Inquiry

PAGE 3

SPEC #:

PATIENT: [REDACTED]

(Continued)

GROSS DESCRIPTION:

The specimen is submitted in five containers.

A. The first specimen submitted in the fresh state is labeled "right breast tissue". The specimen is the right breast, total mastectomy specimen. Suture marks the axillary tail of the breast. There is an ellipse of white skin containing the nipple. This measures 9 x 4.5 cm. The breast itself measures 23 x 22.5 x 6.5 cm. There is a firm mass palpable beneath the surface at approximately the 11 o'clock position. The overlying superficial margin adjacent to the skin is marked with black ink and the deep margin beneath this is marked with blue ink. On cut section the mass measures approximately 4.5 x 3 x 3 cm. The edges are quite difficult to discern and are more easily palpated than visualized. In the center of the specimen there is a small focus of hemorrhage consistent with previous biopsy. A portion of this mass is set aside for tissue banking per protocol. The mass extends to within approximately 1 cm of the overlying superficial margin at approximately 11 o'clock at its closest point. It is located well above the deep margin which is approximately 7 cm beneath the mass. Cut sections through the remainder of the breast tissue reveals some rubbery white parenchymal breast tissue and abundant yellow fat. There are no other nodules or mass identified. Sections are submitted as follows:

- A1 - nipple
- A2-5 - mass in breast
- A6 - mass in breast with adjacent tissue
- A7-9 - non-neoplastic appearing breast tissue

B. The second specimen is received in the fresh state in the operating room and represents two axillary lymph nodes, one measuring 2 x 1.5 x 1 cm and the other measuring 1 x 0.7 x 0.7 cm. The cut sections are hard. A section of each is examined by frozen section. These are submitted in block B1, frozen section control. Additional section of each is submitted in block B2.

C. The third specimen initially in the fresh state is labeled "sentinel node #1" and is a piece of yellow fat measuring 2.5 x 2 x 1 cm with a 1 cm round moderately firm lymph node which is bisected and submitted in toto in block C1.

D. The fourth specimen is labeled "level 3 axillary nodes". The specimen is a piece of yellow fatty tissue measuring 9 x 3 x 1.5 cm with a suture marking the highest level 3 lymph node. There are five additional small lymph nodes. The highest node is submitted in block D1 and the other five lymph nodes are submitted in block D2.

E. The fifth container is labeled "right axillary contents". The specimen is yellow fatty tissue measuring 10 x 9 x 2.5 cm. Within the fatty tissue are several nodules consistent with lymph nodes. The largest of these measures 2 cm in diameter and on cut section is partially replaced by very hard white tissue. A section of each node is submitted in blocks E1 through E4.

MICROSCOPIC DESCRIPTION:

The slides are examined and evaluated.

** CONTINUED ON NEXT PAGE **

RUN DATE:
RUN TIME:
RUN USER:

Specimen Inquiry

PAGE 4

SPEC #:

PATIENT:

(Continued)

INTRAOPERATIVE CONSULTATION:
OPERATING ROOM CONSULTATIONS:

A. Portions of the breast mass are processed for tumor tissue banking per protocol.

B. FROZEN SECTION DIAGNOSIS OF RIGHT AXILLARY LYMPH NODES X2:
- METASTATIC CARCINOMA IN TWO LYMPH NODES

PHOTO DOCUMENTATION

Image .
Image .
Image .
Image .
Image .
Image .

Signed ____ (signature on file) ____

** END OF REPORT **

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
Staging Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Notes	✓	
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer	3/15/13	3/16/13